

UNIVERSITY MEDICAL CENTER
Neurology & Stroke Unit — 7 North

DISCHARGE SUMMARY

Document No: DS-2024-03312	Admission Date: March 8, 2024	Discharge Date: March 15, 2024
Attending Physician: Dr. Sandra Chen, MD, PhD, FAAN		
Patient Name: Rosa Elena Gutierrez-Morales	Date of Birth: October 22, 1955 (Age: 68)	
MRN: UMC-991-0034	Sex: Female	
Insurance: Medicaid	Discharge Disposition: Assisted Living — Casa Esperanza Memory Care	

LANGUAGE / COMMUNICATION NOTE

[!] Patient's primary language is Spanish. English proficiency: limited.

[!] Medical interpreter was used during hospitalization. Facility must arrange Spanish-speaking staff or certified interpreter for all care communication.

[!] Patient's daughter (Maria Gutierrez, 301-555-8847) is designated healthcare proxy and primary contact — Spanish/English bilingual.

PRIMARY DIAGNOSIS

Left middle cerebral artery (MCA) ischemic stroke — acute

Secondary: Atrial fibrillation (new onset, identified during admission), Hypertension, Hyperlipidemia, Obesity (BMI 34)

REASON FOR ADMISSION

Patient presented to ED on 3/8 via EMS with sudden onset right-sided weakness, facial droop, and expressive aphasia at 09:22. NIHSS on arrival: 14. CT head: no hemorrhage. CT angiography: left MCA M2 occlusion. Patient received IV tPA at 10:04 (onset-to-needle: 42 minutes). Mechanical thrombectomy performed at 11:15 with TICI 2b reperfusion. Patient admitted to stroke unit for monitoring and rehabilitation initiation.

HOSPITAL COURSE

Post-thrombectomy course complicated by mild hemorrhagic transformation (HI-1) on MRI day 2 — anticoagulation held x48 hours. Atrial fibrillation identified on telemetry day 2 — cardiology consulted. Apixaban initiated after 72-hour observation per neurologist guidance. Right upper and lower extremity weakness partially improved. Expressive aphasia improving — patient producing single words and short phrases. Dysphagia screen positive — speech therapy initiated, diet modified.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
Apixaban (NEW)	5mg	PO	Twice daily	For AFib — do NOT crush (swallowing difficulty noted)
Aspirin (DISCONTINUED)	—	—	STOPPED	D/C due to apixaban initiation — do not restart
Lisinopril	20mg	PO	Once daily	BP target <130/80 post-stroke
Atorvastatin (INCREASED)	80mg	PO	Nightly	UPTITRATED from 40mg — high-intensity statin

Pantoprazole (NEW)	40mg	PO	Once daily	GI protection with anticoagulation
Metoprolol Tartrate (NEW)	25mg	PO	Twice daily	For rate control — AFib

DYSPHAGIA & DIET

Diet: IDDSI Level 5 (minced and moist) — NO thin liquids.

All liquids must be nectar-thick consistency.

Patient requires assistance and supervision with all meals.

Speech therapy to continue at facility: 5x per week — order placed.

[!] Apixaban is a tablet — patient has dysphagia. Crushing protocol NOT addressed in discharge orders.

[!] Nectar-thick liquid thickener brand/product not specified — facility to confirm with speech therapy.

REHABILITATION ORDERS

Physical therapy: 5x per week — right hemiparesis rehabilitation.

Occupational therapy: 5x per week — ADL retraining.

Speech-language pathology: 5x per week — aphasia and dysphagia.

All therapy orders transmitted to facility.

FOLLOW-UP REQUIRED

1. Neurology (Dr. Chen): 4 weeks post-discharge — 301-555-7700

2. Cardiology (Dr. Kim — AFib management): 2 weeks — 301-555-6612

3. Repeat MRI brain: 3 months

[!] Neurology appointment: NOT scheduled — family to call.

[!] Cardiology appointment: NOT scheduled — family to call.

ALLERGIES

NKDA (No known drug allergies)